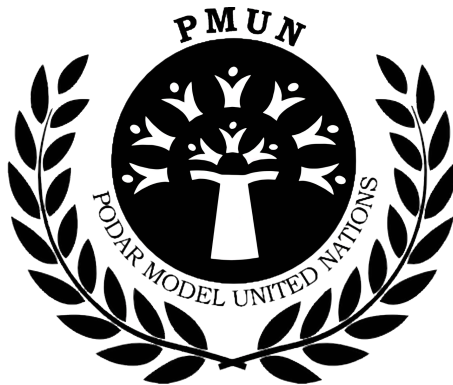




PMUN

BACKGROUND GUIDE - 2

Topic: Impact of Foreign Aid Reductions on Global Child Healthcare

Committee: United Nations Children's Fund (UNICEF)



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A. Understanding the Issue

Many low-income countries depend heavily on foreign aid to support child healthcare, vaccination drives, nutrition programs, and medical infrastructure. Organisations such as UNICEF, WHO, GAVI, and donor countries have historically funded immunisation campaigns, healthcare clinics, and emergency services in these regions.

Recent reductions in foreign aid have created major challenges for healthcare systems already struggling with poverty, conflict, and weak infrastructure. These cuts may lead to vaccine shortages, the closure of rural clinics, reduced access to healthcare, and an increased risk of preventable diseases.

Millions of children rely on externally funded healthcare systems for survival. Without strong healthcare systems and immunisation programs, countries may face increased child mortality, disease outbreaks, and long-term developmental setbacks.

B. Why is this Important Globally?

Child healthcare and immunisation are directly connected to global health security, economic stability, human rights, and sustainable development. In an interconnected world, healthcare crises in one region can quickly affect other parts of the world through disease outbreaks, migration pressures, and economic instability. Ensuring access to healthcare for children is therefore not only a regional concern for Sub-Saharan Africa but also an important global responsibility.

Public Health

Vaccination programs are among the most effective public health measures and protect millions of children from preventable diseases such as measles, polio, and cholera. Strong immunisation systems help reduce child mortality rates, improve life expectancy, and prevent healthcare systems from becoming overwhelmed during outbreaks. However, reductions in foreign aid may disrupt vaccine distribution and weaken already fragile healthcare systems in many African countries.

Global Health Security

Weak healthcare systems increase the risk of infectious diseases spreading across borders. The COVID-19 pandemic highlighted how health emergencies in one region can quickly become global crises. Maintaining strong immunisation programs and healthcare infrastructure is therefore important not only for the region itself but also for international disease prevention and emergency preparedness.

Economic Development

Healthy children are more likely to attend school, develop skills, and contribute positively to the economy in the future. Strong healthcare systems reduce long-term medical costs and improve productivity, while disease outbreaks can place significant financial pressure on governments and families. As a result, investments in child healthcare are closely linked to long-term economic growth and social stability.

Human Rights

Access to healthcare is recognised internationally as a basic human right. The United Nations Convention on the Rights of the Child states that every child has the right to healthcare, nutrition, and medical support. However, many children in low-income and conflict-affected regions still lack access to basic healthcare services due to poverty, weak infrastructure, and dependence on foreign aid.

Sustainable Development Goal 3 (SDG 3)

SDG 3 aims to “Ensure healthy lives and promote well-being for all at all ages” by 2030. This includes reducing child mortality, improving access to vaccination, strengthening healthcare systems, and preventing disease outbreaks. Achieving SDG 3 will require international cooperation, sustainable healthcare financing, and stronger local healthcare systems, especially in regions that remain heavily dependent on external aid.

C. Key Statistics & Trends

- At least 14 million children across the globe face disruptions to nutrition services in 2025 due to funding cuts.
- 2.5 million additional child deaths projected from aid cessation between 2025-2030 (According to the Lancet study.)
- 16.8 million pregnant women annually losing USAI-supported maternal care.
- 479 health facilities at risk of closure globally.

*These statistics show the impact of the reduction in external aid clearly.

D. Learn About Your Committee – UNICEF

What Does UNICEF Do?

- Protects children's rights globally
- Supports healthcare, nutrition, education, and immunisation
- Provides emergency humanitarian aid
- Works with governments and NGOs to improve child welfare

Powers:

- Can recommend international child welfare policies
- Can coordinate vaccination campaigns
- Can work with governments and donor organisations

Limitations:

- Cannot force countries to implement policies
- Depends heavily on voluntary funding
- Faces logistical challenges in conflict zones

Past Actions:

- Led global polio eradication campaigns
- Supported COVID-19 vaccine distribution
- Expanded nutrition and WASH programs

E. UN Frameworks for Children Health and Nutrition

I. Convention on the Rights of the Child (1989)

The Convention on the Rights of the Child (CRC) is one of the most widely ratified international human rights treaties in history. It recognises that every child has the right to healthcare, nutrition, education, protection, and development. The treaty encouraged governments to improve child welfare systems and expand access to healthcare services, especially for vulnerable children.

Key Features:

1. Recognises healthcare as a fundamental child right
2. Promotes child nutrition, vaccination, and protection
3. Encourages governments to prioritise child welfare policies

Why it Matters:

1. Established international standards for child healthcare
2. Influenced global health and child protection programs
3. Strengthened international focus on child survival and development

Limitations:

1. Enforcement depends on national governments
2. Many low-income countries struggle with funding and implementation
3. Conflict and political instability continue to affect child healthcare access

II. Millennium Development Goals (MDGs) (2000–2015)

The Millennium Development Goals were a set of global development targets adopted by the United Nations in 2000. Two major goals focused on reducing child mortality and improving maternal health. The MDGs increased global attention on vaccination programs, disease prevention, nutrition, and healthcare access in developing countries, especially in Africa.

Key Features:

1. Focused on reducing child mortality
2. Encouraged global cooperation on healthcare
3. Increased donor funding for immunisation and disease prevention
4. Improved monitoring of global health progress

Why it Matters:

1. Helped reduce child deaths in several countries
2. Expanded immunisation campaigns
3. Increased awareness about healthcare inequality

Limitations:

1. Progress was uneven across regions
2. Many healthcare systems remained dependent on foreign aid
3. Long-term healthcare infrastructure development remained limited

III. Sustainable Development Goal 3 (SDG 3)

SDG 3 aims to “Ensure healthy lives and promote well-being for all at all ages” by 2030. It is part of the broader Sustainable Development Goals adopted by the United Nations in 2015. SDG 3 is especially important because many countries continue to face challenges such as high child mortality, infectious diseases, malnutrition, weak healthcare infrastructure, and limited access to healthcare.

Key Targets:

1. Reduce child and maternal mortality
2. End epidemics such as HIV/AIDS, malaria, and tuberculosis
3. Improve vaccination coverage
4. Strengthen healthcare systems and workforce capacity
5. Increase access to affordable medicines and healthcare services

Why it matters:

1. Connects healthcare to long-term development
2. Encourages international cooperation and funding
3. Promotes equal healthcare access

Challenges:

1. Dependence on foreign aid
2. Shortage of trained healthcare workers
3. Weak infrastructure in rural and conflict-affected regions
4. Funding gaps and unequal healthcare access

IV. GAVI – The Vaccine Alliance

GAVI, the Vaccine Alliance, was established in 2000 to improve access to vaccines in low-income countries. It works in partnership with UNICEF, WHO, governments, and donor organizations to support immunization programs and vaccine financing. GAVI has played a major role in expanding vaccine coverage across Sub-Saharan Africa.

Key Features:

1. Provides financial support for vaccines
2. Supports immunisation infrastructure and healthcare systems
3. Works with international organisations and governments
4. Helps lower vaccine costs for developing countries

Why it Matters:

1. Increased vaccine access for millions of children
2. Reduced outbreaks of preventable diseases
3. Strengthened global immunisation programs

Limitations:

1. Many countries remain dependent on donor funding
2. Vaccine delivery remains difficult in remote areas
3. Funding shortages can disrupt immunisation programs

Insight: Delegates can discuss whether developing countries should prioritise local vaccine manufacturing to reduce dependence on international aid.

V. COVAX Initiative

The COVAX Initiative was launched during the COVID-19 pandemic to ensure fair and equal global access to vaccines, particularly for low-income and developing countries. The initiative was co-led by WHO, UNICEF, GAVI, and CEPI and became one of the largest global vaccine-sharing efforts in history.

Key Features:

1. Focused on equitable vaccine distribution
2. Supported low- and middle-income countries
3. Encouraged international cooperation during the pandemic
4. Delivered billions of vaccine doses globally

Why it matters:

1. Helped improve vaccine access during a global health crisis
2. Demonstrated the importance of international healthcare cooperation
3. Supported healthcare systems in vulnerable countries

Limitations:

1. Wealthier countries often secured vaccines faster
2. Supply shortages affected distribution
3. Funding in Healthcare inequalities remained visible during the pandemic

Insight: Delegates may debate whether global funding in healthcare systems are truly equitable during international emergencies.

VI. UNICEF Immunisation and Health Programs

UNICEF has led several major healthcare and immunization programs in low-income countries, including polio eradication campaigns, nutrition support, cold-chain transportation systems, and emergency healthcare services. The organization also supports healthcare worker training, maternal healthcare, sanitation programs, and awareness campaigns.

Key Features:

1. Supports child vaccination campaigns
2. Provides emergency medical and nutrition aid
3. Helps maintain cold-chain vaccine systems
4. Works in conflict zones and remote areas

Why it matters:

1. Protects millions of vulnerable children
2. Strengthens healthcare access in underserved regions
3. Supports long-term child welfare and development

Limitations:

1. Heavily dependent on donor funding
2. Difficulties operating in conflict regions
3. Healthcare demand often exceeds available resources

F. Country Positions

Countries involved in global healthcare and foreign aid have different priorities, responsibilities, and challenges. While donor countries often focus on global health security, diplomacy, and economic interests, many low-income nations have challenges in healthcare access, sustainable funding, and dependence on external financial assistance. Understanding these positions is important for delegates when forming alliances, negotiating resolutions, and proposing realistic long-term solutions.

I. United States

The United States has historically been one of the world's largest contributors to global healthcare and humanitarian assistance through organisations such as USAID, the Centers for Disease Control and Prevention (CDC), and partnerships with UNICEF, WHO, and GAVI. American funding has supported vaccination drives, HIV/AIDS treatment programs, maternal healthcare, nutrition programs, and emergency disease response initiatives across all low-income countries globally.

Programs such as PEPFAR (President's Emergency Plan for AIDS Relief) have played a major role in reducing HIV/AIDS-related deaths and strengthening healthcare systems in African countries.

Key Interests:

1. Preventing the spread of infectious diseases globally
2. Maintaining global health security
3. Supporting humanitarian and development goals
4. Strengthening diplomatic influence and international partnerships

Challenges:

1. Political debates over foreign aid spending
2. Balancing domestic priorities with international commitments
3. Concerns about long-term aid dependence

II. United Kingdom

The United Kingdom has traditionally been a major supporter of global health and development initiatives through Official Development Assistance (ODA). The UK has contributed significantly to organisations such as GAVI, UNICEF, and WHO, supporting immunisation campaigns, maternal healthcare, and nutrition programs in low-income countries. However, recent reductions in foreign aid budgets have raised concerns about the future of several healthcare and child welfare programs.

Key Interests:

1. Supporting global health and development goals
2. Strengthening international cooperation
3. Preventing global disease outbreaks
4. Supporting child healthcare and vaccination initiatives

Challenges:

1. Economic pressures and budget limitations
2. Domestic political debates over aid spending
3. Ensuring aid effectiveness and accountability

III. India

India has emerged as an important healthcare and pharmaceutical partner for many developing countries, including nations in Sub-Saharan Africa. India is one of the world's largest vaccine producers and played a significant role during the COVID-19 pandemic by supplying affordable vaccines and medicines through initiatives such as Vaccine Maitri. India also promotes South-South cooperation and capacity-building partnerships in healthcare and medical training.

Key Interests:

1. Expanding healthcare diplomacy
2. Strengthening trade and diplomatic ties with African nations
3. Supporting affordable medicine and vaccine access
4. Increasing global influence through humanitarian cooperation

Challenges:

1. Balancing domestic healthcare needs with international commitments
2. Managing large population healthcare demands
3. Ensuring equitable vaccine distribution

IV. China

China has significantly expanded its healthcare diplomacy and infrastructure investments across Africa over the past two decades. Through bilateral agreements, medical assistance programs, and the Belt and Road Initiative, China has supported hospital construction, donations of medical equipment, training for healthcare workers, and vaccine distribution in several African countries. China also increased its vaccine diplomacy efforts during the COVID-19 pandemic.

Key Interests:

1. Expanding diplomatic and economic influence in Africa
2. Strengthening trade and political partnerships
3. Supporting international development cooperation
4. Increasing global leadership presence

Challenges:

1. High transition costs
2. Balancing energy affordability with sustainability
3. Grid stability during the transition
4. Managing the phase-out of coal-dependent regions

V. Sub-Saharan African Nations

Many countries in Sub-Saharan Africa continue to depend heavily on international funding for child healthcare, vaccination programs, nutrition support, and disease prevention initiatives. Governments across the region are increasingly calling for stronger healthcare infrastructure, local vaccine manufacturing, and sustainable financing systems to reduce long-term dependence on foreign aid.

At the same time, many countries face serious challenges including poverty, political instability, conflict, shortages of healthcare workers, weak transportation systems, and limited domestic healthcare budgets.

Key Interests:

1. Expanding healthcare access for children and vulnerable populations
2. Strengthening local healthcare infrastructure
3. Increasing local vaccine and medicine production
4. Reducing long-term aid dependence
5. Improving healthcare worker training and retention

Challenges:

1. Limited domestic funding and resources
2. Weak healthcare systems in rural regions
3. Conflict and humanitarian crises
4. Dependence on international donors and NGOs

G. Case Studies

1. Somalia – Nutrition Centre Closures

Foreign aid reductions forced nutrition centres to shut down, placing children at risk.

Source: <https://www.unicef.org/>

2. Malawi – Vaccine Shortages

Malawi faced healthcare challenges due to shortages in vaccines and infrastructure.

Source: <https://www.who.int/>

3. Nigeria – Maternal and Child Health Challenge

Aid cuts affected maternal healthcare services and immunisation programs.

Source: <https://www.gavi.org/>

4. Rwanda – Digital Health Systems

Rwanda invested in digital vaccination tracking systems.

Source: <https://www.unicef.org/rwanda/>

5. Afghanistan: One of the most aid-dependent healthcare systems in the world. Following aid freezes post-2021 and recent US cuts, UNICEF-supported mobile health and nutrition units were drastically reduced. Millions of children face acute malnutrition with almost no domestic healthcare infrastructure to fall back on.

6. Haiti: Political instability combined with donor fatigue has repeatedly disrupted child vaccination programs. Cholera re-emerged in 2022, partly due to the collapse of health infrastructure – a direct consequence of reduced international support.

H. List of possible Agendas

1. Should foreign aid for children healthcare come under a condition compelling recipient countries to build domestic healthcare capacity, or such conditions worsen the economy and healthcare of recipient countries?
2. Is there a possibility of a globally agreed notice period or a transition plan before a major donor withdraws funding in children healthcare to prevent the sudden collapse of the healthcare infrastructure in a low-income country?
3. Can other countries take the initiative to formally take over responsibility to create a multination framework for funding and stakeholding in organisations like UNICEF and GAVI to counter reduced aid from traditional Western donor countries?

I. References & Recommended Reading

- UNICEF: <https://www.unicef.org/>
- WHO: <https://www.who.int/>
- GAVI: <https://www.gavi.org/>
- UN SDG 3: <https://sdgs.un.org/goals/goal3>
- World Bank Health: <https://www.worldbank.org/en/topic/health>
- COVAX: <https://www.gavi.org/covax-facility>

- UK Foreign, Commonwealth & Development Office:
<https://www.gov.uk/government/organisations/foreign-commonwealth-development-office>
- Ministry of External Affairs (India): <https://www.mea.gov.in/>
- Serum Institute of India: <https://www.seruminstitute.com/>
- China International Development Cooperation Agency: <http://en.cidca.gov.cn/>
- WHO Africa-China Health Cooperation: <https://www.afro.who.int/>
- African Union CDC: <https://africacdc.org/>